



SUDEEKSHA HEALTH CARE

We care for long lives...

#135/5, Muthanallur Circle, Anekal Taluk, Bangalore - 560099.

Mob: 6366620121 / 9113552234

Patient Name : MISS. SREEPARNA
Ref Doctor : SELF
Client Name: BLR00184-Sudeeksha Health
Care

Age / Gender : 25 years/Female
Client Patient ID : 267592
Status : **Final Report**

Sample Registered : 07/12/2025, 10:41 AM
Sample Received : 07/12/2025, 02:32 PM
Reported : 07/12/2025, 04:38 PM

Test Description	Value(s)	Units	Biological Ref Range	Method
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SPAN HEALTH PACKAGE -
PLATINUM

HEMATOLOGY



HBA1C (GLYCOSYLATED HEMOGLOBIN), WHOLE BLOOD

Sample Type : Edta Whole Blood

HBA1C	5.3	%	4.30 - 6.40	HPLC
MPG - MEAN PLASMA GLUCOSE	105.41	mg /dL	70 - 140	-

Result	Hemoglobin A1c(%)
Normal	less than 5.7%
Prediabetic	5.7% to 6.4%
Diabetes	6.5% or higher

(Ref: ADA (American Diabetic Association) guidelines 2016)

Remarks: Kindly correlate clinically.

****END OF HEMATOLOGY****



Signature

Dr. Tuhinika AB
M.B.B.S, MD Pathology
KMC Reg: 8163-KTK



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Age / Gender: 25 years/Female
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Status: Final Report

Sample Registered: 07/12/2025, 10:41 AM
Sample Received: 07/12/2025, 02:49 PM
Reported: 07/12/2025, 04:06 PM

Test Description	Value(s)	Units	Biological Ref Range	Method
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SPAN HEALTH PACKAGE -
PLATINUM

BIOCHEMISTRY



GLUCOSE FASTING, PLASMA

Sample Type: Fluoride Plasma F

GLUCOSE FASTING

108

mg/dL

70 - 99

SPECTROPHOTOMETRY,
HEXOKINASE

Test Performed on Roche cobas pure

As per American Diabetic Association, (ADA) 2018 Guidelines Fasting Plasma Glucose Value (in mg/dl)

Interpretation

- 70 - 99 Normal
- 100 - 125 Pre-Diabetic
- ≥ 126 Diabetes mellitus

It is recommended that fasting plasma glucose be repeated on TWO separate occasions or fasting plasma glucose with HbA1c should be done to confirm the diagnosis of Diabetes mellitus.

Remarks:

Kindly correlate clinically.

****END OF BIOCHEMISTRY****



T. Srinivas
Dr. T. Srinivas
M.B.B.S, MD Pathology
KMC Reg: 0163-KTK



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Age / Gender : 25 years/Female

Sample Registered : 07/12/2025, 10:41 AM

Ref Doctor : SELF

Client Patient ID : 267592

Sample Received : 07/12/2025, 03:35 PM

Client Name: BLR00184-Sudeeksha Health Care

Status : Final Report

Reported : 07/12/2025, 04:06 PM

Test Description	Value(s)	Units	Biological Ref Range	Method
SPAN HEALTH PACKAGE - PLATINUM		BIOCHEMISTRY		 15253410011
LIPID PROFILE, SERUM				
Sample Type : Serum				
CHOLESTEROL TOTAL SERUM	141	mg/dL	<200	CHOLESTEROL ESTERASE
HDL	35.2	mg/dL	Major Risk: <40 Negative Risk: > 60	DIRECT ENZYMATIC
TRIGLYCERIDES	156	mg /dL	0 - 150	GPO- POD
NON HDL CHOLESTROL	105.80	mg/dL	2 - 130	CALCULATED
CHOLESTEROL LDL	74.60	mg/dL	Optimal: <100 Near/above optimal: 100 - 129 Borderline High: 130 - 159 High: 160 - 189 Very High: ? 190	CALCULATED
VLDL CHOLESTROL	31.20	mg/dL	0 - 30	CALCULATED
TOTAL CHOLESTROL / HDL RATIO	4.01	Ratio	3.3 - 4.4 LOW RISK 4.5 - 7.0 AVERAGE RISK 7.1 - 11.0 MODERATE RISK >11.0 HIGH RISK	CALCULATED
LDL/HDL CHOLESTROL RATIO	2.12	Ratio	2.5 - 3.5	CALCULATED

Interpretation

TRIGLYCERIDES CAN SHOW MARKED VARIATION DEPENDING ON PREVIOUS DAY DIET INTAKE. 12 HRS FASTING IS MANDATORY BEFORE TESTING FOR LIPID PROFILE SPECIALLY FOR TRIGLYCERIDE VALUES. IN CASE, LIPID PROFILE IS DONE IN NON FASTING STATE, THEN ANY ABNORMAL VALUE, ESPECIALLY FOR TRIGLYCERIDES MUST BE RETESTED ON OVERNIGHT FASTING SAMPLE. CALCULATED LDL & VLDL VALUES MAY BE HIGHLY VARIABLE IF NON FASTING SAMPLES ARE TESTED.

FRIEDEWALD FORMULA FOR CALCULATING LDL IS NOT VAILD IF TRIGLYCERIDES IS > 400 AND IN PATIENTS WITH TYPE 3 DYSLIPOPROTEINEMIA.

Remarks:

Kindly correlate clinically.

END OF BIOCHEMISTRY



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Patient Name: MISS. SREEPARNA

Age / Gender: 25 years/Female

Sample Registered: 07/12/2025, 10:41 AM

Ref/Dctor: SELF

Client Patient ID: 267582

Sample Received: 07/12/2025, 02:35 PM

Client Name: BLR001154-Sudeeksha Health
Care

Status: Final Report

Reported: 07/12/2025, 04:17 PM

Test Description	Value(s)	Units	Biological Ref Range	Method
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SPAN HEALTH PACKAGE - PLATINUM

BIOCHEMISTRY



LIVER PROFILE, SERUM

Sample Type: Serum

BILIRUBIN TOTAL	0.14	mg/dL	0.30 - 0.90	SPECTROPHOTOMETRY, DIAZO METHOD
BILIRUBIN DIRECT	0.11	mg/dL	0.00 - 0.30	SPECTROPHOTOMETRY, DIAZO METHOD, Photometry
BILIRUBIN INDIRECT	0.03	mg/dL	0.20 - 1.00	CALCULATED
ALANINE AMINOTRANSFERASE (SGPT)	30.6	U/L	14 - 59	SPECTROPHOTOMETRY, UV WITH PYRIDOXAL-5-PHOSPHATE
ASPARTATE AMINOTRANSFERASE (SGOT)	32.9	U/L	15 - 37	SPECTROPHOTOMETRY, UV WITH PYRIDOXAL-5-PHOSPHATE
ALKALINE PHOSPHATASE	82.3	U/L	35 - 104	SPECTROPHOTOMETRY, PNP AMP KINETIC, Photometry
PROTEIN TOTAL	7.76	g/dl	6.0 - 8.2	SPECTROPHOTOMETRY, BIURET, Photometry
ALBUMIN	4.22	g/dl	3.5 - 5.2	BROMORESOL PURPLE (BCP)
GLOBULIN	3.54	g/dl	2.0 - 4.10	Calculated
ALBUMIN / GLOBULIN RATIO	1.19	-	1.0 - 2.1	-
GAMMA GLUTAMYL TRANSFERASE	16.0	U/L	5-36	Photometry

Test is performed over Roche Cobas Pure

Remarks:

Kindly correlate clinically.

"END OF BIOCHEMISTRY"



Signature

Dr. Subashini, MD
NABL, NCP Pathology
KMC, Bangalore



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Age / Gender : 25 years/Female
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Status : Final Report

Sample Registered : 07/12/2025, 10:41 AM
Sample Received : 07/12/2025, 03:35 PM
Reported : 07/12/2025, 04:21 PM

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SPAN HEALTH PACKAGE -
PLATINUM

BIOCHEMISTRY



IRON PROFILE

Sample Type : Serum

IRON	8.8	µg/dL	33 - 193	SPECTROPHOTOMETRY, FERENE, Photometry
% SATURATION	2.69	%	13 - 45	-
UIBC	318.7	µg/dL	135 - 392	-
TOTAL IRON BINDING CAPACITY	327.50	µg/dL	250 - 450	Calculated

Interpretation

Iron found in the blood is mainly present in the hemoglobin of the RBCs. Its role in the body is mainly in the transportation. Iron is absorbed in the Small Intestine, and bound to a globulin in the plasma called Transferrin and transported to the Bone Marrow for the formation of Hemoglobin. Increased serum levels are found in Hemolytic Anemias, Hepatitis, Lead and Iron poisoning. Decreased serum levels are found in Anemias caused by Iron Deficiency due to insufficient intake or absorption of Iron, chronic blood loss, late pregnancy and Cancer TIBC The serum TIBC varies in disorders of Iron metabolism. In Iron-deficiency Anemia the TIBC is elevated and the transferrin saturation is lowered to 15% or less. Low serum Iron associated with low TIBC is characteristic of the anemia of chronic disorders, malignant tumors, and infections.

Remarks:

Kindly correlate clinically.

****END OF BIOCHEMISTRY****



Signature
Dr. Tejaswini Ali
M.B.B.S, MD Pathology
KMC Reg: 0163 KTK

SUDEEKSHA HEALTH CARE

Year	Age	Sex	Height (cm)	Weight (kg)	Body Mass Index (kg/m ²)	Waist Circumference (cm)	Hip Circumference (cm)	Waist-Hip Ratio
1990	18	M	175	75	24.2	85	95	0.89
1995	23	M	178	85	27.5	90	100	0.90
2000	28	M	180	95	29.6	95	105	0.90
2005	33	M	182	105	32.7	100	110	0.91
2010	38	M	185	115	33.5	105	115	0.91
2015	43	M	188	125	35.1	110	120	0.92
2020	48	M	190	135	37.4	115	125	0.92



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Sample Registered : 07/12/2025, 10:41 AM
Sample Received : 07/12/2025, 03:35 PM
Reported : 07/12/2025, 05:15 PM

Test Description	Value(s)	Units	Biological Ref Range	Method
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SPAN HEALTH PACKAGE -
PLATINUM

IMMUNOLOGY



VITAMIN B12

Sample Type : Serum

VITAMIN B12

234

pg/mL

211 - 911

CLIA

Interpretation

Uses of Vitamin B12 assay: • Investigation of macrocytic anaemia • Work up of deficiencies seen in Megaloblastic Anemia • Assistance in Diagnosis of CNS Disorders • Evaluation of Alcoholism • Evaluation of Malabsorption syndrome
Limitation: • The evaluation of Macrocytic Anemia requires simultaneous measurement of both Vitamin B12 and folate levels. • Patients taking B12 supplementation may have misleading results

Test performed on Roche Cobas Pure

:

AGE

Premature infants

1-3

4-6

7-11

12-18

REFERENCE RANGE

365-1568

345-1154

330-1236

312-1237

328-1185

Remarks:

Kindly correlate clinically.

END OF IMMUNOLOGY



Signature
Dr. Tathiana AB
M.B.B.S, MD Pathology
KMC Reg. 8163 KYS



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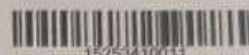
Age / Gender : 25 years/Female
Client Patient ID : 267592
Status : Final Report

Sample Registered : 07/12/2025, 10:41 AM
Sample Received : 07/12/2025, 05:45 PM
Reported : 07/12/2025, 06:28 PM

Test Description	Value(s)	Units	Biological Ref Range	Method
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SPAN HEALTH PACKAGE -
PLATINUM

CLINICAL PATHOLOGY



15253410011

URINE ROUTINE EXAMINATION.

Sample Type : Urine

PHYSICAL EXAMINATION

COLOR	PALE YELLOW
TRANSPARENCY	SLIGHTLY HAZY

CHEMICAL EXAMINATION

pH	6.0	-	4.6 - 8.0	DIPSTIX STRIP
SPECIFIC GRAVITY	1.005	-	1.003 - 1.030	DIPSTIX STRIP
PROTEIN	NOT DETECTED	-	NOT DETECTED	DIPSTIX STRIP
GLUCOSE	NOT DETECTED	-	NOT DETECTED	DIPSTIX STRIP
KETONES	NOT DETECTED	-	NOT DETECTED	DIPSTIX STRIP
BLOOD	NOT DETECTED	-	NOT DETECTED	DIPSTIX STRIP
UROBILINOGEN	NORMAL	-	NORMAL	DIPSTIX STRIP
BILE SALTS AND PIGMENTS	NOT DETECTED	-	NOT DETECTED	DIPSTIX STRIP
NITRITE	NEGATIVE	-	NEGATIVE	-
LEUCOCYTE ESTERASE	NEGATIVE	-	NEGATIVE	-

MICROSCOPIC EXAMINATION

PUS CELLS/WBCS	3-5	/HPF	0 - 5	MICROSCOPY
EPITHELIAL CELLS	2-3	/HPF	0 - 5	MICROSCOPY
RED BLOOD CELLS	NIL	/HPF	NIL	MICROSCOPY
CRYSTALS	NOT DETECTED	-	NOT DETECTED	MICROSCOPY
CASTS	NOT DETECTED	-	NOT DETECTED	MICROSCOPY
BACTERIA	PRESENT	-	NOT DETECTED	MICROSCOPY

Others

Remarks:

Kindly correlate clinically.

END OF CLINICAL PATHOLOGY



Signature
Dr. Tahmina Ali
M.B.B.S, MD Pathology
KMC Reg: 0163 KTK



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Ref Doctor : SELF Client Patient ID : 267592 Sample Received : Dec 07, 2025, 03:35 p.m.
Client Name: BLR00184-Sudeeksha Health Care Status : Final Report Reported : Dec 07, 2025, 04:06 p.m.

Test Description	Value(s)	Units	Biological Ref Range	Method
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SPAN HEALTH PACKAGE -
PLATINUM

BIOCHEMISTRY



PHOSPHORUS INORGANIC, SERUM

Sample Type : Serum

PHOSPHORUS

3.14

mg /dL

2.5 -4.5

PHOSPHOMOLYBDATE

CHILDREN reference range :4.0 -7.0 mg/dl

Remarks:

Kindly correlate clinically.

****END OF BIOCHEMISTRY****



Processed at : Diagnostica Span Pvt Ltd

No.42, 9th B Main Road, Muni Reddy Layout HRBR Layout, Banaswadi, Bangalore -

560043

Tehide
Dr. Tehide Ali
M.B.B.S, MD Pathology
KMC Reg: 8163 KTK



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Sample Registered : 07/12/2025, 10:41 AM

Sample Received : 07/12/2025, 02:32 PM

Reported : 07/12/2025, 06:16 PM

Test Description

Value(s)

Units

Biological Ref Range

Method



Signature
Dr. Tamsinda Ali
M.B.B.S, MD Pathology
KMC, Bap. 9163 ETL



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Care

Test Description	Value(s)	Units	Biological Ref Range	Method
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SPAN HEALTH PACKAGE -
PLATINUM

IMMUNOLOGY



25-HYDROXY VITAMIN D, SERUM

Sample Type : Serum

25-HYDROXY VITAMIN D

45.6

ng/mL

<20 DEFICIENCY
20.0 - 30.0
INSUFFICIENCY
30.0 - 100.0
SUFFICIENCY
>100.0 TOXICITY

CMIA

Interpretation

Uses for Vitamin D assay: • Diagnosis of Vitamin D deficiency • Differential Diagnosis of causes of Rickets and Osteomalacia •
Monitoring Vitamin D replacement therapy • Diagnosis of Hypervitaminosis D

LIMITATION: Various methods are available for measuring circulating concentrations of 25-OH vitamin D. The studies report reasonable correlation between methods, but with significant differences, the reasons for which are not well understood. Vitamin D values must be interpreted within the clinical context of each patient.

Test performed on Roche Cobas Pure

Remarks:

Kindly correlate clinically.

****END OF IMMUNOLOGY****



Signature
Dr. Vidushi A3
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KMC, Bap. BHO KTK



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Status : **Final Report**

Sample Registered : 07/12/2025, 10:41 AM
Sample Received : 07/12/2025, 03:35 PM
Reported : 07/12/2025, 05:14 PM

Test Description	Value(s)	Units	Biological Ref Range	Method
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SPAN HEALTH PACKAGE -
PLATINUM

IMMUNOLOGY



15253410011

THYROID PROFILE, TOTAL, SERUM

Sample Type : Serum

TRI-IODO THYRONIN (T3)	1.38	ng/mL	0.6 - 1.81	CMIA
THYROXIN (T4)	5.53	ug/dl	3.20 - 14.1	CMIA
THYROID STIMULATING HORMONE	3.82	µIU/mL	0.35 - 5.50	CMIA

Test Performed on Roche cobas pure

Normal ranges for infants and children

Age	T3 (ng/mL)	T4 (µg/dL)	TSH (µIU/mL)
Cord blood	0.15 - 0.75	7.8 - 13.1	2.2 - 10.7
1 to 4 days	1.0 - 7.4	9.3 - 20.9	1.0 - 39.0
4 to 30 days	1.05 - 3.87	8.0 - 21.8	1.7 - 9.10
1 to 12 months	1.05 - 2.45	7.2 - 15.7	0.7 - 6.40
1 to 5 years	1.05 - 2.69	6.4 - 13.5	0.64 - 6.27
6 to 10 years	0.94 - 2.41	6.0 - 12.8	0.64 - 6.27

Reference Ranges for Pregnancy

1st Trimester	0.10 - 2.50
2nd Trimester	0.20 - 3.00
3rd Trimester	0.30 - 3.00

Interpretation:

Assay results should be interpreted in context to the clinical condition and associated results of other investigations. Abnormal thyroid test findings often found in critically ill clients should be repeated after the critical nature of the condition is resolved. The production, circulation, and disposal of Thyroid hormone are altered throughout the stages of pregnancy. Hyperthyroidism (overactive thyroid) : Hyperthyroidism (overactive Thyroid) occurs when your thyroid gland produces too much of the hormone Thyroxine. Hyperthyroidism can accelerate your body's metabolism, causing unintentional weight loss and a rapid or irregular heartbeat. Hypothyroidism (underactive thyroid) : Hypothyroidism (underactive thyroid) is a condition in which your Thyroid gland doesn't produce enough of certain crucial hormones. Hypothyroidism may not cause noticeable symptoms in the early stages. Over



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Status : Final Report

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Sample Received : 07/12/2025, 02:32 PM
Reported : 07/12/2025, 05:47 PM

Test Description	Value(s)	Units	Biological Ref Range	Method
SPAN HEALTH PACKAGE - PLATINUM		HEMATOLOGY		 15253410011
<u>COMPLETE BLOOD COUNT.</u>				
Sample Type : Edta Whole Blood				
HEMOGLOBIN	<u>10.9</u>	g/dl	12 - 15	CN-Free HB
RBC COUNT	3.90	10 ⁶ /uL	3.80 - 4.80	Optical
HEMATOCRIT	34.0	%	36 - 46	Calculated
MCV	87.2	fL	80 - 100	Measured
MCH	28.1	pg	27 - 32	Calculated
MCHC	32.2	g/dl	31.5 - 34.5	Calculated
RDW-CV	14.5	%	11.60 - 15.4	Measured
TOTAL LEUCOCYTE COUNT	8.40	10 ³ /uL	04 - 10	Flowcytometry
NEUTROPHILS	64.9	%	40 - 80	Flowcytometry
LYMPHOCYTES	24.5	%	20-40	Flowcytometry
MONOCYTES	7.0	%	2.0 - 10.0	Flowcytometry
EOSINOPHILS	3.2	%	1.0 - 6.0	Flowcytometry
BASOPHILS	0.40	%	<2.0	Flowcytometry/Calculated
ABSOLUTE NEUTROPHIL COUNT	5.45	10 ³ /uL	2.00 - 7.00	Calculated
ABSOLUTE LYMPHOCYTE COUNT	2.06	10 ³ /uL	01 - 03	Calculated
ABSOLUTE MONOCYTE COUNT	0.59	10 ³ /uL	0.20 - 1.0	Calculated
ABSOLUTE EOSINOPHIL COUNT	0.27	10 ³ /uL	0.02 - 0.50	Calculated
ABSOLUTE BASOPHIL COUNT	0.03	10 ³ /uL	0.02 - 0.10	Calculated
PLATELET COUNT	<u>130</u>	10 ³ /uL	150 - 450	Optical

COMMENT

Remarks:

Giant platelets are noted.
Kindly correlate clinically.



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Reported : 07/12/2025, 04:05 PM

Test Description	Value(s)	Units	Biological Ref Range	Method
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SPAN HEALTH PACKAGE -
PLATINUM

BIOCHEMISTRY



KIDNEY FUNCTION TEST, SERUM.

Sample Type : Serum

UREA	20.1	mg/dL	10.0 - 50.0	SPECTROPHOTOMETRY, UREASE-GLDH
BLOOD UREA NITROGEN	9.39	mg/dL	07 - 18	Calculated
URIC ACID	1.95	mg/dL	2.6 - 6.2	SPECTROPHOTOMETRY, URICASE
CREATININE	0.53	mg/dL	0.5 - 1.0	SPECTROPHOTOMETRY, JAFTE-KINETIC
BUN / CREATININE RATIO	17.72	Ratio	5.0 - 20.0	Calculated
CALCIUM	9.3	mg/dL	8.2 - 10.1	SPECTROPHOTOMETRY, O-CRESOLPHTHALEIN COMPLEXONE (OCPC) REACTION
SODIUM	139.1	mmol/L	135 - 145	DIRECT ISE
POTASSIUM	4.36	mmol/L	3.50 - 5.50	DIRECT ISE
CHLORIDE	102.9	mmol/L	98 - 111	ISE, INDIRECT

Test is performed over Roche Cobas Pure

Remarks:

Kindly correlate clinically.

****END OF BIOCHEMISTRY****



Signature

Dr. Tejaswini A
M.B.B.S, MD Pathology
KMC Reg-8163 K25